

Anterior complications and aqueous obstruction

Uveitis can affect transparency and drainage as well as the primary inflamed tissue.

Structure predicts complication

Structure	Consequence
Cornea	Endothelial damage causes oedema; prolonged disease may cause band keratopathy
Iris-lens interface	Posterior synechiae may block the pupil and bow iris forwards
Drainage angle	Peripheral anterior synechiae impair aqueous outflow
Lens	Inflammation and steroid exposure can cause cataract

Separate the two adhesion sites

Posterior synechiae join iris to lens and can obstruct posterior-to-anterior chamber flow. Peripheral anterior synechiae involve the angle and obstruct drainage directly. Both can raise pressure, but the initial blocked pathway differs. Steroid treatment can also elevate pressure despite an open angle. Monitor pressure and lens clarity during follow-up rather than assuming that control of anterior chamber cells excludes future visual complications.

Posterior inflammation and central blur

A quiet-looking anterior eye may still have vitreous or macular disease causing visual loss.

Localise the residual symptom

What is the dominant posterior finding?

Vitreous leukocytes and floaters

Intermediate uveitis is centred on the vitreous/pars plana and may be painless.

Active chorioretinal inflammation

Posterior uveitis involves the choroid and retina.

Central blur and distortion

Look for cystoid macular oedema from leakage through inflamed vessels.

Protect vision beyond the anterior chamber

Cystoid macular oedema and cataract are important sight-threatening complications. OCT helps assess macular swelling when distortion or central blur persists. Inflammation can also cause disc oedema, retinal vascular complications or detachment. Anatomical classification follows the predominant inflammatory site: anterior, intermediate and posterior inflammation without a predominant site is panuveitis. Do not use absence of redness or pain to exclude posterior inflammation.